

Clinical Staff Feedback Form

Automated PAR Index Calculation System

 All responses are strictly confidential

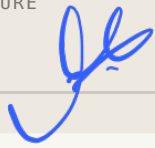
PROJECT
CO2060 · 2nd year

CONTACT
e22036@eng.pdn.ac.lk

RESPONDENT
Clinical / Academic Staff

Purpose: This form collects your expert clinical opinion on the usability, accuracy, and practical value of our web-based system for automated PAR Index calculation. Your input will directly inform our development priorities for 2nd year and shape the system's readiness for real-world clinical deployment.

RESPONDENT SIGNATURE



DATE

2024/06/15

1 Respondent Information

CLINICAL ROLE

Registrar in Orthodontics

YEARS OF CLINICAL EXPERIENCE

10 years

INSTITUTION / HOSPITAL

FDS - WOP

EMAIL ADDRESS (OPTIONAL – FOLLOW-UP ONLY)

pradeepmityranga@gmail.com

Have you used any similar digital orthodontic assessment tools before?

☐

Yes

☒

No

☐

Unsure

IF YES, PLEASE NAME THE SYSTEM(S)

2

General Usability

Rate your level of agreement · 1 = Strongly Disagree · 5 = Strongly Agree

Statement	1 Str. Disagree	2 Disagree	3 Neutral	4 Agree	5 Str. Agree
The overall interface is intuitive and easy to navigate.	☐	☐	☐	☒	☐
I was able to understand the system's output without additional guidance.	☐	☐	☐	☒	☐
The workflow matches how I approach PAR scoring in clinical practice.	☐	☐	☐	☒	☐
The system minimises unnecessary complexity for clinical staff.	☐	☐	☐	☒	☐
I would feel comfortable using this system with minimal prior training.	☐	☐	☐	☒	☐

3

Feature-Specific Evaluation

Rate the quality / usefulness of each feature · 1 = Very Poor · 5 = Excellent

Feature	1 Very Poor	2 Poor	3 Fair	4 Good	5 Excellent
File Upload — ease of uploading 3D models (.STL / .OBJ) and file validation feedback	☐	☐	☐	☒	☐
Automated Landmark Detection — accuracy and reliability of AI-identified anatomical points	☐	☐	☐	☐	☐
3D Visualisation Viewer — rendering quality, zoom / rotate / pan responsiveness	☐	☐	☐	☐	☐
Manual Override Tool — ability to adjust and correct AI-placed landmark positions	☐	☐	☐	☐	☐
PDF Report Generation — clarity, completeness, and clinical usefulness of the output report	☐	☐	☐	☐	☒

4.1

How easy was it to upload a 3D dental model and initiate PAR calculation?

Very Difficult

Difficult

Neutral

Easy

Very Easy

4.2

How would you rate the 3D model viewer's performance and visual clarity?

Very Poor

Poor

Acceptable

Good

Excellent

4.3

How useful is the manual landmark override feature for clinical practice?

Not useful at all

Slightly useful

Moderately useful

Very useful

Essential

4.4

How suitable are the generated PDF reports for inclusion in clinical records?

Not suitable

Needs major revision

Needs minor revision

Mostly suitable

Fully suitable

4.5

How confident are you in the clinical accuracy of the AI-computed PAR scores? (1 = Not at all confident · 10 = Extremely confident)

1

2

3

4

5

6

7

8

9

10

Not at all confident

Extremely confident

4.6

What concerns, if any, do you have about integrating AI-based PAR scoring into clinical workflows? (Tick all that apply)

☐ Accuracy and reliability of AI landmark detection☐ Data privacy and patient confidentiality☒ System errors causing incorrect treatment decisions☒ Liability and accountability for AI-generated results☐ Over-reliance on automation reducing clinical skill☐ Resistance or lack of trust from the clinical team

☐ No major concerns

5 Efficiency & Time Savings

5.1

Compared to manual PAR scoring, how much time do you estimate this system saves per case?

No time saved

Less than 5 min

5–10 minutes

10–15 minutes

More than 15 min

5.2

Which stage of the digital workflow felt the most time-consuming?

File Upload

AI Landmark Detection

Manual Correction / Override

Report Export

None — all stages were fast

6.1

In approximately what percentage of cases did you feel the need to use the Manual Override tool?

0% — AI was always accurate

Less than 25%

Around 50%

More than 75%

Used override on every case

6.2

Are there specific dental landmarks where the AI consistently underperforms? (Tick all that apply)

- | | |
|--|---|
| ☐ Upper anterior segment / overjet | ☐ Lower anterior segment |
| ☐ Right buccal occlusion (molar relationship) | ☐ Left buccal occlusion (molar relationship) |
| ☐ Overbite / vertical overlap | ☐ Centreline / midline deviation |
| ☐ No consistent underperformance noticed | |
-

6.3

How closely did the AI-generated PAR score match your own manual calculation for the same model? (1 = Completely different · 10 = Perfect match)

1

2

3

4

5

6

7

8

9

10

Completely different

Perfect match

7.1

How would you rate the system's responsiveness when manipulating the 3D model (zoom, rotate, pan)?

☐ Very laggy / unusable☐ Noticeable lag☒ Acceptable☐ Smooth☐ Very smooth / responsive

7.2

If a file upload failed or a calculation crashed, was the error feedback from the system helpful enough to resolve the issue?

☐ Did not encounter any errors☐ Error message was confusing☐ Partially helpful☐ Mostly helpful☐ Fully helpful — resolved immediately

8.1

Did you experience any lag or rendering issues on the device you were using?

☐ No issues at all☐ Minor lag only☐ Moderate lag☐ Significant lag☐ System was unusable on my device

Device used:

☐ Laptop☐ Desktop PC☐ Tablet☐ Other

IF 'OTHER', SPECIFY DEVICE

8.2

How important is it for this system to sync directly with your existing Patient Management System (PMS)?

☐ Not important☐ Slightly important☐ Moderately important☒ Very important☐ Essential — would not adopt without this

9.1

Does the PDF report provide enough visual evidence (e.g. landmark screenshots) to justify the PAR score to a third party or auditor?

☐ Definitely not☐ Probably not☐ Unsure☒ Probably yes☐ Definitely yes

9.2

How effective would this report be as a visual aid when explaining treatment needs to a patient?

☐ Not effective at all☐ Slightly effective☒ Moderately effective☐ Very effective☐ Extremely effective

10

Overall Assessment

10.1

Overall, how would you rate this system for clinical use?

Very Poor

Poor

Fair

☒ Good

Excellent

10.2

Would you recommend this system to colleagues or other orthodontic institutions?

Definitely not

Probably not

Unsure

☒ Probably yes

Definitely yes

10.3

Which features do you consider most valuable for clinical practice? (Tick all that apply)



Automated landmark detection



3D model visualisation



Manual override and correction



Longitudinal patient tracking



Downloadable PDF reports



Model versioning and reproducibility



Multi-institution / cloud access

11.1

What specific improvements would most increase your confidence in using this system clinically?

AS detection part & machine learning for automatic point detection must be improved.

11.2

Are there any clinical or institutional barriers that might prevent adoption of this system at your facility? (e.g. IT infrastructure, ethical clearance, data governance, training requirements)

Man power to obtain a sufficient data bank for ML part.

11.3

If you could automate one additional part of your clinical workflow (e.g. cephalometric analysis, treatment outcome prediction), what would be your top priority?

Ceph analysis.

11.4

Any additional comments, suggestions, or features you would like to see in future versions?